

OPPORTUNITY REVIEW

DIRECT-TO- EMPLOYER GTM STRATEGY

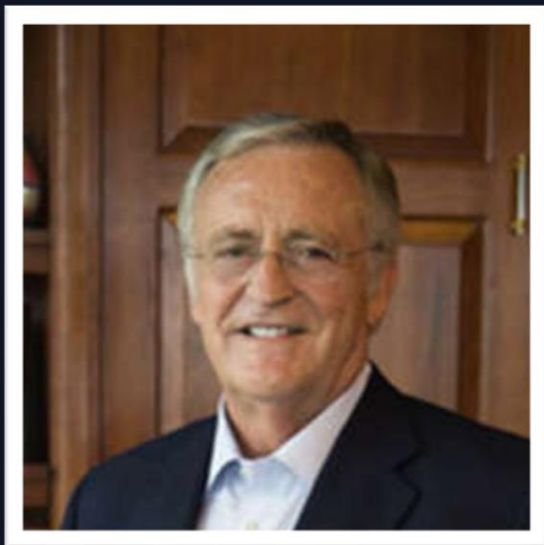
Exploring a WakeMed Health and Hospitals DTE Health Plan





KEY BENEFIT ADMINISTRATORS

- Founded in 1979, headquartered in Indianapolis
- Largest Independently-Owned Full-Service Third-Party Administrator
- Specialize in Administration of Self-Funded Plans and Population Health Management
- Leader in the Direct-to-Employer Movement
- We Know and Understand the Market
- Experience Working with All Professional Teams: Agents, Networks, MGAs
- The Key Family of Companies is a Vertically Integrated group of benefit related organizations
 - 3,000+ Corporate Clients
 - 3 Million Members under Management
 - 700 Employees



Our Founding and Heritage:

46 Years of Striving for Honesty in Healthcare

Founded Key Benefit Administrators in 1979

After co-founding a life insurance agency in 1973 and Key Life Insurance in 1976, Larry Dust launched KBA in 1979 as a third-party benefits administrator focused on honest healthcare delivery.

Sold to Blue Cross/Blue Shield in 1985

Strategic partnership with BCBS of Indiana (then part of the Associated Group) provided capital and market access while maintaining operational independence.

Reacquired KBA in 1992

Larry bought the business back from BCBS in 1992, returning it to complete independence and patient-focused mission alignment.

Built the Largest Independent TPA in the U.S.

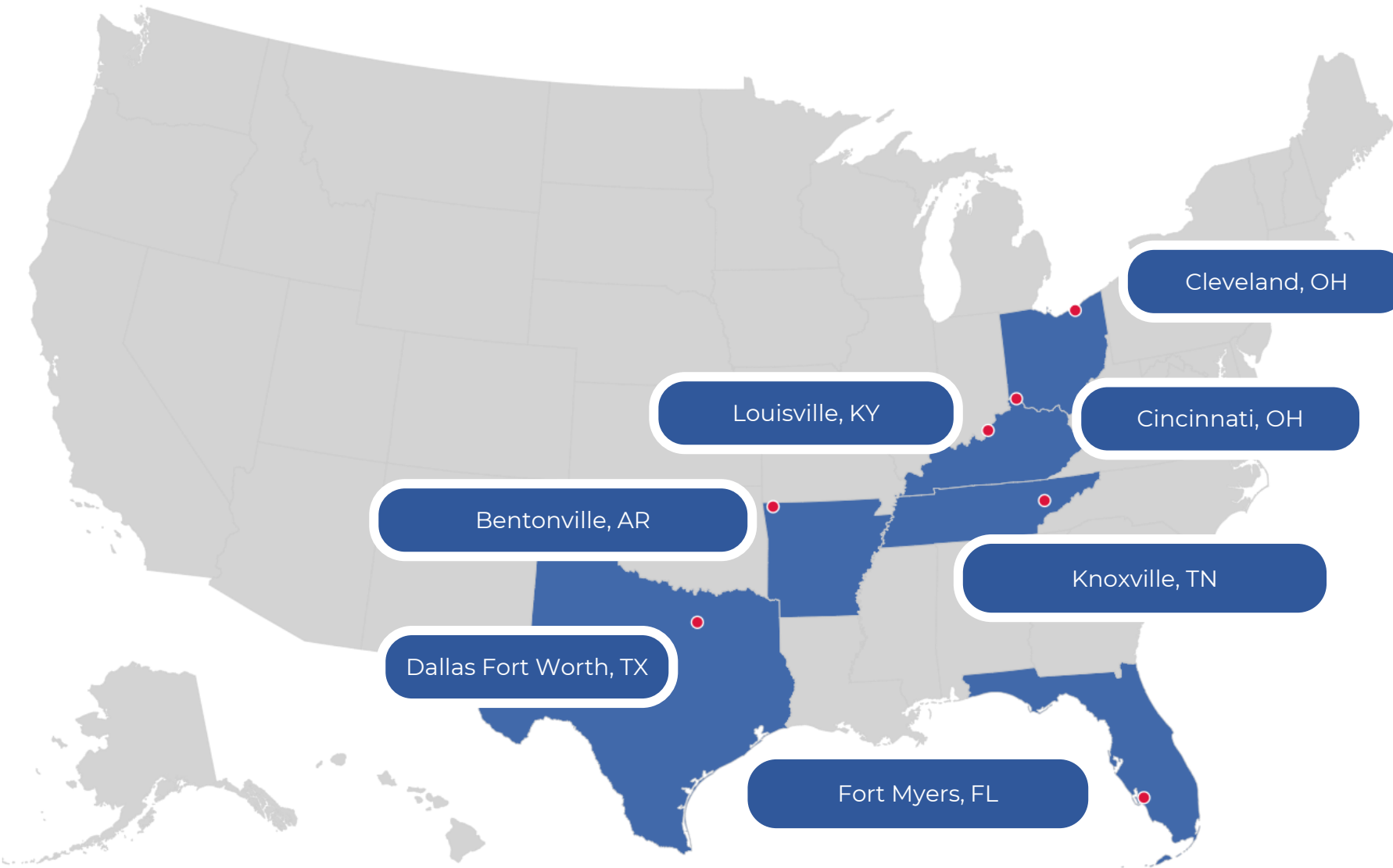
Under his leadership since the buyback, KBA has become the country's largest independently owned third-party administrator, managing millions of members across diverse markets.

Never Forgotten Who Gives Healthcare

KBA has never participated in reference-based pricing, does not take refunds, and delivers healthcare at 11% below market rates. Our commitment remains with providers and patients, not profit extraction.

Key Benefit Administrators

MARKET LEADER IN PRIVATE LABELED DTE PLANS



DIRECT-TO-EMPLOYER (DTE)

Your Health Plan
Focused on a Direct
Relationship with Self-
Funded Employers in
Your Market



YOU Pick the Name of the Health Plan



YOU Determine the Fees Charged



YOU Practice the Medicine



YOU Receive the Profits



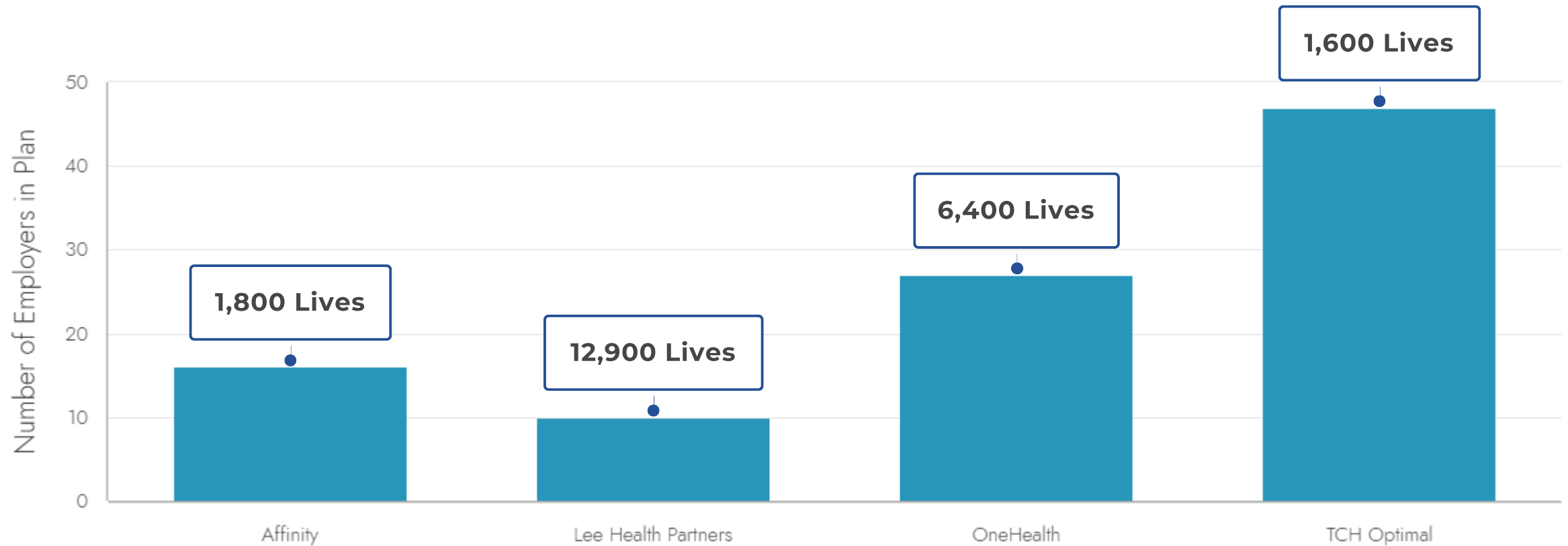
WE Make it Successful



Direct-to-Employer Strategy: POWERFUL EMPLOYER VALUE

- Reduced medical spend through lower unit costs and better integrated care
- “At the table” with WakeMed Health and Hospitals
- Aligned financial incentives
- Complete transparency for unlimited visibility into health benefit plan expenses and cost drivers
- Aggressive stop-loss premiums and policy terms
- Increased employee satisfaction
- Improved cash flow & stabilization of future medical cost trend

KBA PLAN GROWTH IN THE FIRST TWO YEARS



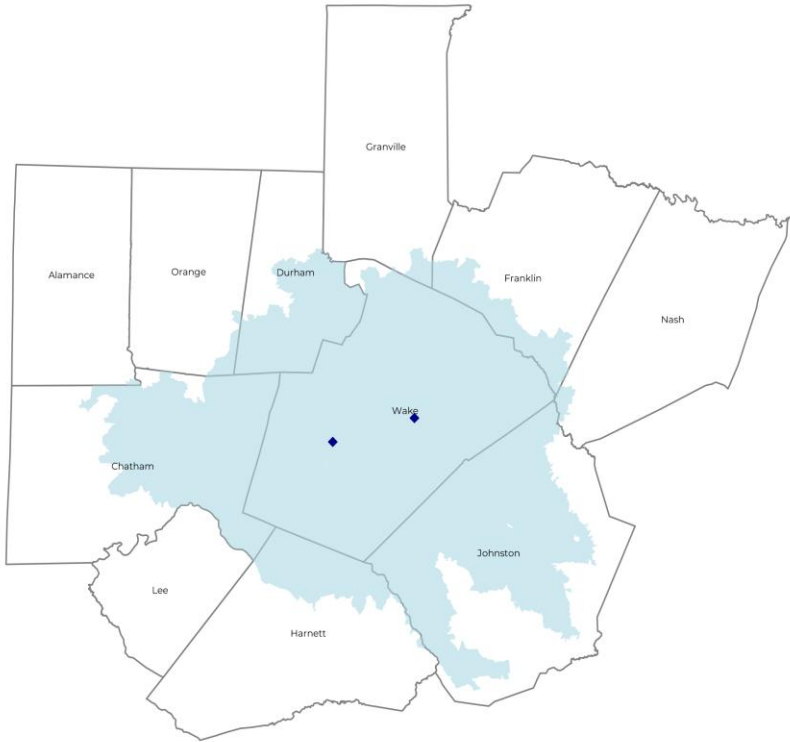
An average of 25 employer groups in plan within the first two years!

SIGNIFICANT OPPORTUNITY FOR PLAN GROWTH

Researched for the below highlighted service area

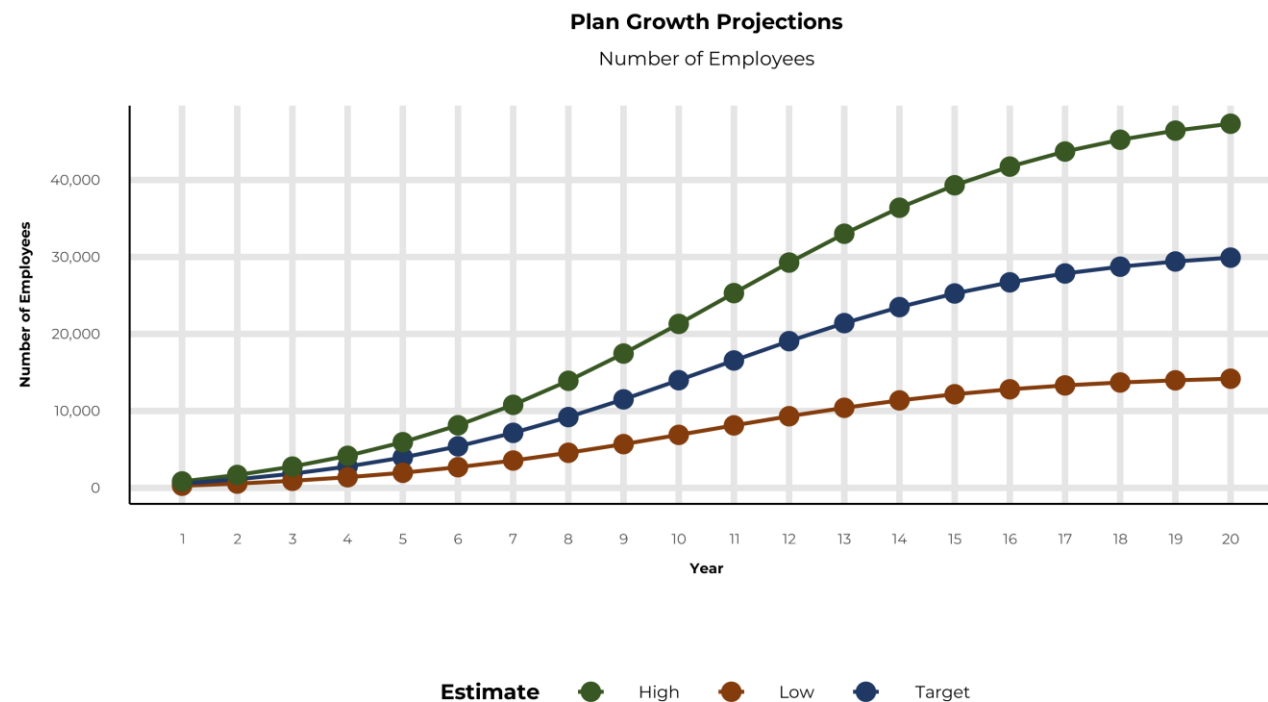
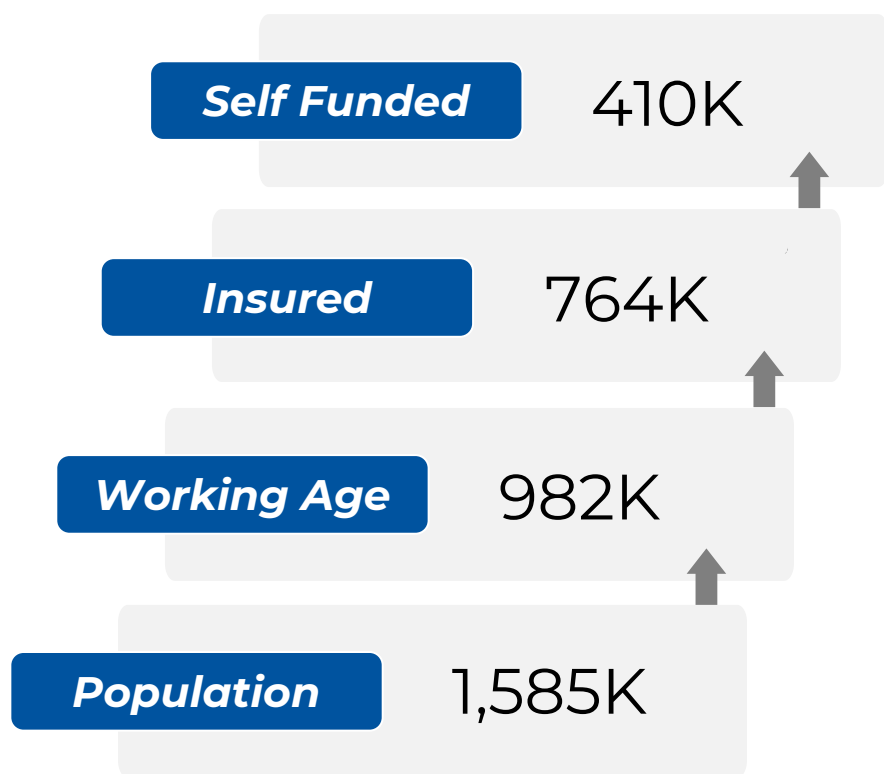
Target Market

2,200+ Employers ➤ 410,000+ Employees ➤ 840,000+ Members



	Employer Size Group (# of Employees)					
Commercial Employers With Headquarters In Catchment Area	50-100	101-200	201-400	401-1,000	1,001-3,000	3,000+
	1,132	497	306	223	67	61
Total Employees at Employers Headquartered in Catchment Area	50-100	101-200	201-400	401-1,000	1,001-3,000	3,000+
	84,900	74,600	91,800	156,100	134,000	366,000
Total Members with Employers Headquartered in Catchment Area	50-100	101-200	201-400	401-1,000	1,001-3,000	3,000+
	152,800	134,300	165,200	281,000	241,200	658,800

PLAN POTENTIAL GROWTH PATHS



Estimates includes the WakeMed Health and Hospitals service area in Chatham, Durham, Franklin, Harnett, Johnston, Orange, and Wake Counties

GROWING POWERFUL, LASTING PROFITABILITY

Early Stage

Gaining Name Recognition, Early Growth

New Revenue:

\$13.47M

New Profit:

\$3.63M

Growth Stage

Strong Reputation, Strong Growth

New Revenue:

\$79.91M

New Profit:

\$22.53M

Mature Stage

High Profitability, Market Saturation

New Revenue:

\$163.48M

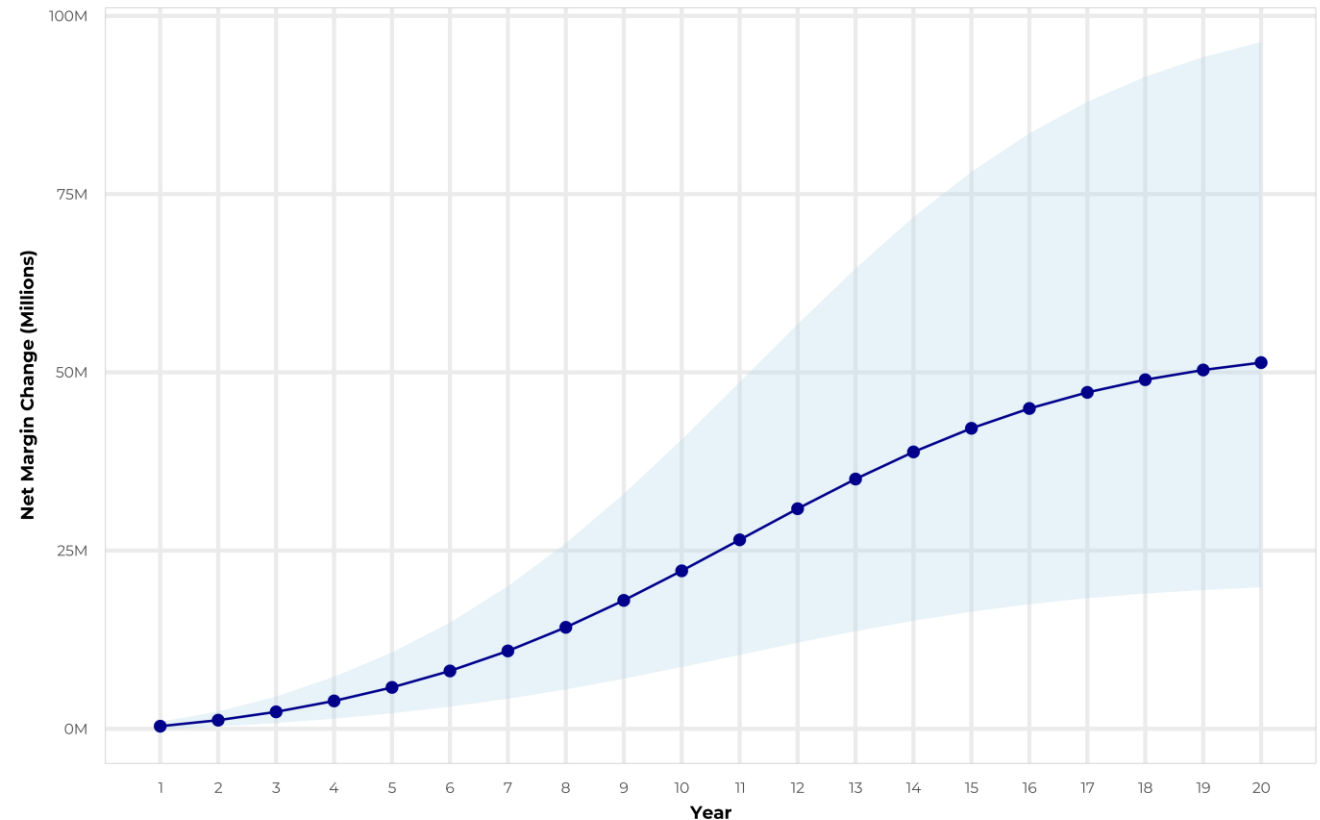
New Profit:

\$46.25M

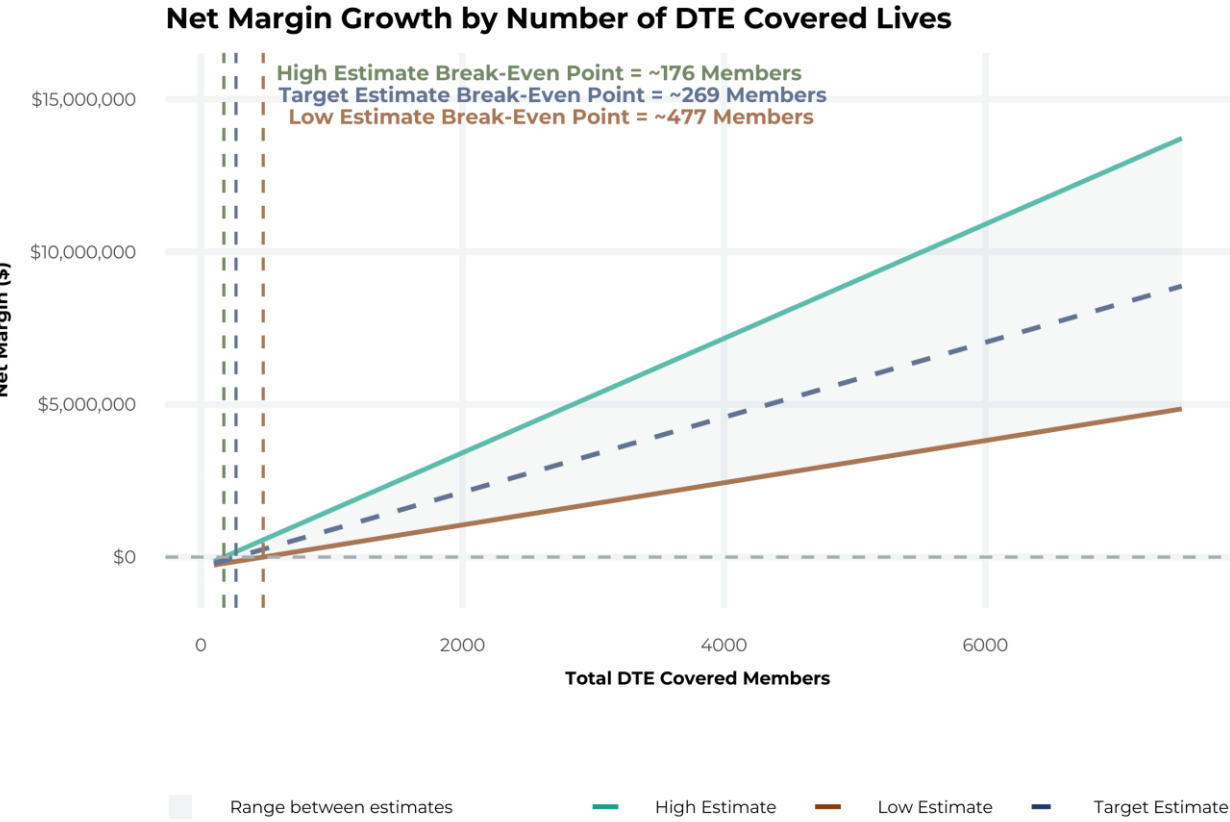
Net Margin Change Over Years

Variation by Member Growth With Target Estimate Assuming

50%+ Within-Plan Steerage and a Commercial Margin of 28%



MODEL ASSUMPTIONS AND BREAK EVEN



Patient Steerage Assumptions

KBA currently estimates that WakeMed Health and Hospitals has 24% inpatient market share, and 8% outpatient market compared to hospitals/clinics in the WakeMed Health and Hospitals service area (see slide 8). The DTE tiered structure will create immediate steerage, with additional steerage over time from patient bonus benefits and the patient convenience card (see slide 33 for estimates of target steerage).

Low	High
10% below target	10% above target

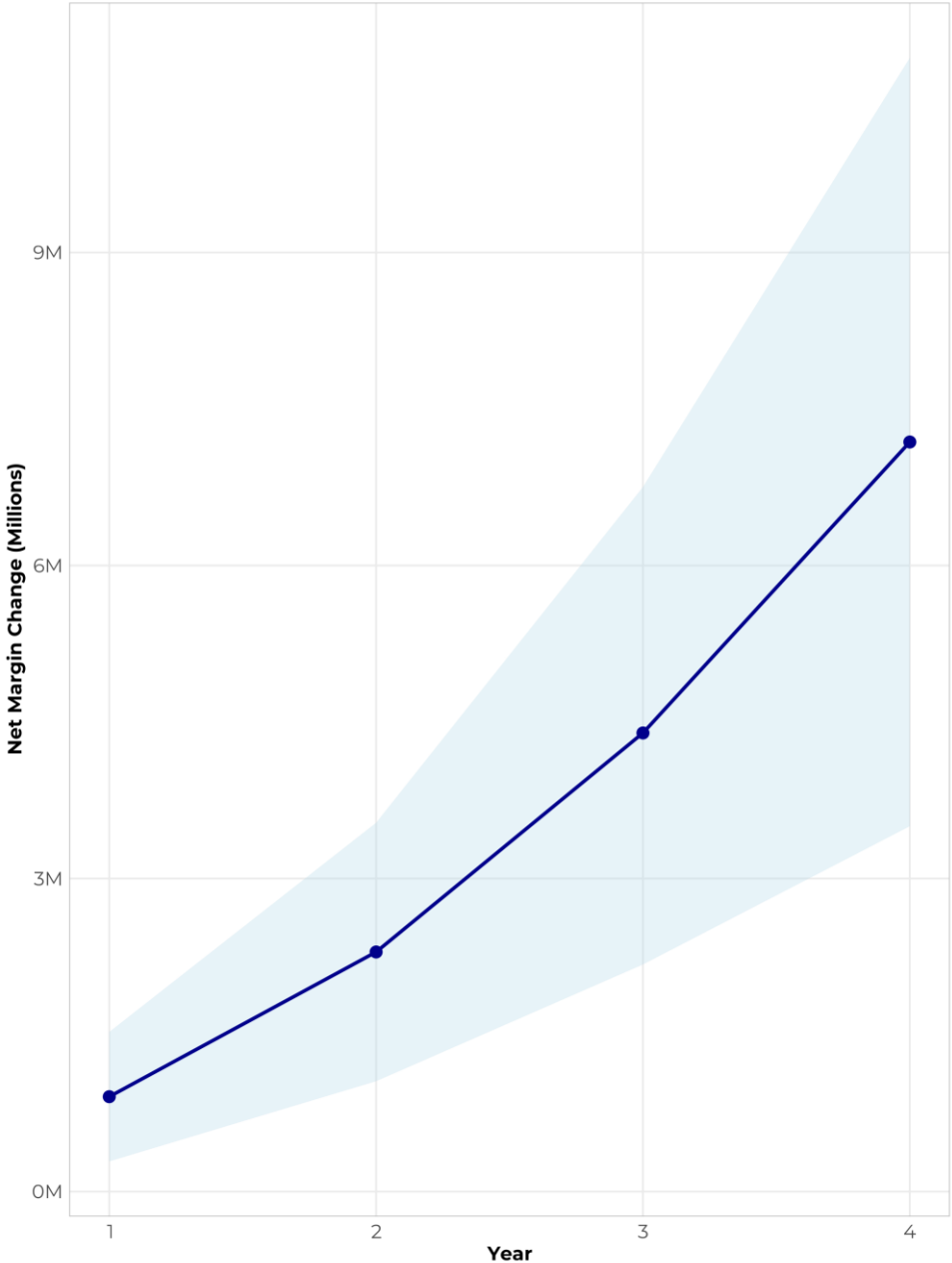
Commercial Margin Assumptions

KBA observes a WakeMed Health and Hospitals 35% commercial margin. Conservatively, we model the DTE to be even with commercial rates given prices 10% below market, but with a 10 percentage-point increase through friendlier billing practices and improved patient collections with the patient card.

Low	Target	High
23%	28%	33%

FOUR-YEAR PROJECTIONS

Net Margin Change Over Years
Variation by Member Growth With Target Estimate Assuming
50%+ Within-Plan Steerage and a Commercial Margin of 28%



Target Member Growth				
Metric	Year 1	Year 2	Year 3	Year 4
Number of Members	1,000	2,000	3,300	5,000
Incremental Margin	1,227.6K	2,452.4K	4,535.8K	7,303.9K
Network Access Fee	12.0K	24.0K	39.6K	60.0K
Implementation Fee (One-Time)	-150.0K	0.0K	0.0K	0.0K
Sales and Marketing Costs	-180.0K	-180.0K	-180.0K	-180.0K
Net Margin Change	909.6K	2,296.4K	4,395.4K	7,183.9K

High Member Growth				
Metric	Year 1	Year 2	Year 3	Year 4
Number of Members	1,500	3,000	5,000	7,500
Incremental Margin	1,841.4K	3,678.6K	6,872.4K	10,955.9K
Network Access Fee	18.0K	36.0K	60.0K	90.0K
Implementation Fee (One-Time)	-150.0K	0.0K	0.0K	0.0K
Sales and Marketing Costs	-180.0K	-180.0K	-180.0K	-180.0K
Net Margin Change	1,529.4K	3,534.6K	6,752.4K	10,865.9K

Low Member Growth				
Metric	Year 1	Year 2	Year 3	Year 4
Number of Members	500	1,000	1,700	2,500
Incremental Margin	613.8K	1,226.2K	2,336.6K	3,652.0K
Network Access Fee	6.0K	12.0K	20.4K	30.0K
Implementation Fee (One-Time)	-150.0K	0.0K	0.0K	0.0K
Sales and Marketing Costs	-180.0K	-180.0K	-180.0K	-180.0K
Net Margin Change	289.8K	1,058.2K	2,177.0K	3,502.0K

DIRECT-TO-EMPLOYER STRATEGY:

ONLY AVAILABLE TO *PREMIER* HEALTH SYSTEMS



STRONG Brand Quality

EXTENSIVE Patient Access

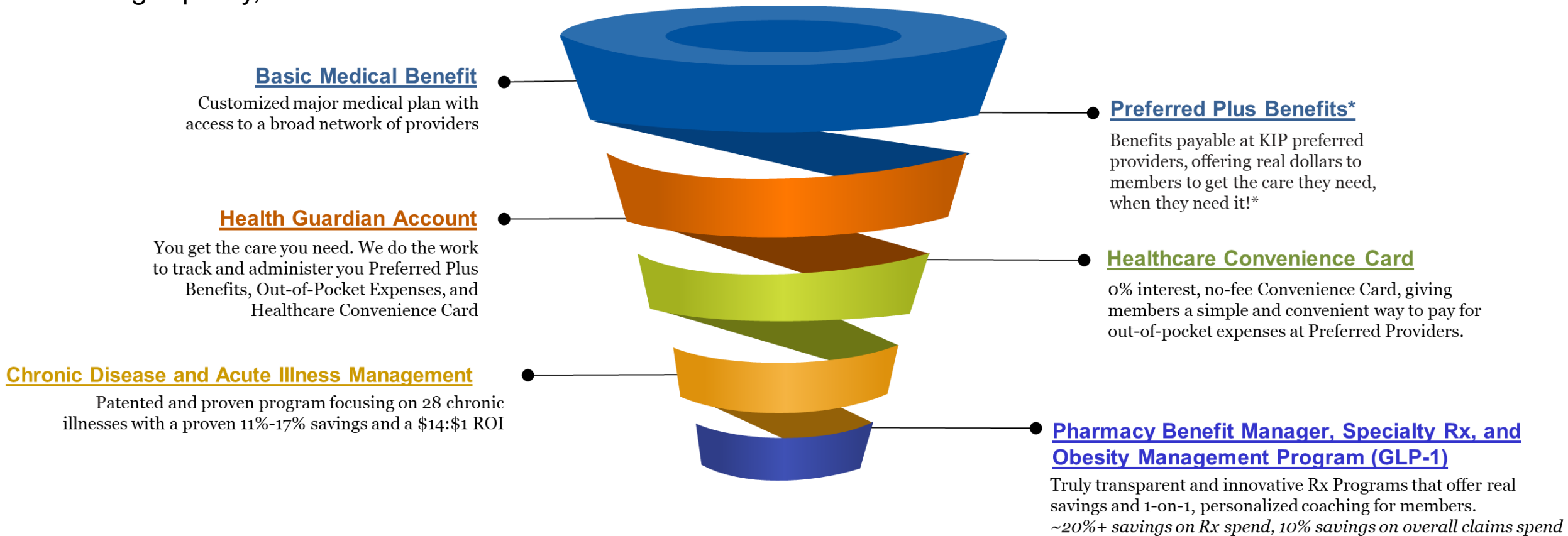
AFFORDABLE Value Pricing

EXCELLENT Care Quality

The Key Incentive Plan

Affordability Without Compromise

The **Key Incentive Plan** is a one-of-a-kind solution, empowering employers to take back control of their healthcare programs, that offers freedom of choice when choosing providers in your local community, while incenting members to choose high-quality, cost-effective care.



Weight Management (GLP-1s) that Don't Break the Bank

The GLP-1 medication market has experienced explosive growth, fundamentally reshaping healthcare spending across America. Prime Therapeutics reports that per-member-per-month spend has tripled by March 2025, with these breakthrough medications now representing approximately 15% of total pharmacy spend. More than 1 in 50 members filed a GLP-1 claim in Q1 2025 alone, demonstrating unprecedented demand for these life-changing treatments.

The Challenge

Traditional GLP-1 medications cost between \$1,089-\$1,349 monthly, creating monthly, creating unsustainable budget pressures for employers while limiting employee access to essential treatments.

Our Solution

Key Benefits secures generic GLP-1 medications for just \$249 per month, enabling comprehensive workforce health while maintaining strict budget control and financial stability.

The Impact

Deliver a healthier population, increased employee satisfaction, and sustainable costs that support long-term organizational success and success and employee wellbeing.

Proven Results: \$7M Annual Savings

One major city spending \$11 million annually on GLP-1 medications achieved \$7 million in cost savings through our comprehensive approach. This dramatic reduction demonstrates how strategic sourcing combined with clinical support creates sustainable, scalable solutions for large employers facing escalating pharmaceutical costs.

Beyond Medication: Comprehensive Care

Our program extends far beyond cost-effective medication access. Members receive specialized coaching twice monthly, focusing on maintaining muscular strength, engaging in moderate exercise, and developing sustainable lifestyle changes that support long-term weight maintenance after GLP-1 treatment completion.

270

Health Conditions

Created or exacerbated by obesity according to Cleveland Clinic research

\$8.5K

Additional Spend

Average annual healthcare costs above normal for obese individuals (KFF)

✔ **Transform Your Healthcare Strategy:** Join forward-thinking employers who are providing comprehensive weight management solutions while protecting their bottom line. Our approach combines clinical excellence with fiscal responsibility, creating a win-win for organizations and their valued employees.

DTE Covered Members Financial Analysis

Target Estimate Assuming 50%+ Within-Plan Steerage and a Commercial Margin of 28%.

DTE Members	Pre-DTE Revenue	Pre-DTE Margin	DTE Revenue	DTE Incr. Revenue	DTE Total Margin	DTE Incr. Margin	Net Margin Less DTE Fees
100	\$ 138K	\$ 39K	\$ 572K	\$ 434K	\$ 160K	\$ 122K	\$ -207K
500	\$ 691K	\$ 194K	\$ 2,862K	\$ 2,171K	\$ 801K	\$ 608K	\$ 284K
1,000	\$ 1,383K	\$ 387K	\$ 5,724K	\$ 4,341K	\$ 1,603K	\$ 1,216K	\$ 898K
2,500	\$ 3,457K	\$ 968K	\$ 14,311K	\$ 10,854K	\$ 4,007K	\$ 3,039K	\$ 2,739K
10,000	\$ 13,827K	\$ 3,872K	\$ 57,242K	\$ 43,415K	\$ 16,028K	\$ 12,156K	\$ 11,946K
20,000	\$ 27,655K	\$ 7,743K	\$ 114,484K	\$ 86,830K	\$ 32,056K	\$ 24,312K	\$ 24,222K

- NOTE: Please see detailed financial analysis and assumptions used starting on page 27.
- Financial projections do not include 340B contribution margin, which could represent \$136 PMPM by optimizing 340B through Rx plan design, WakeMed Health and Hospitals's approved site(s) with network integration, and development of WakeMed Health and Hospitals's proprietary PBM program.

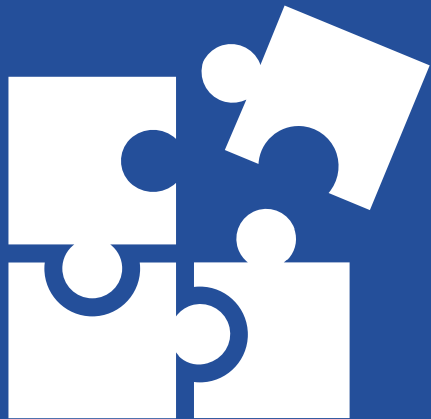
INTEGRATION WITH EPIC EHR SYSTEM

**KBA shares
evidence-based
standards of care for
27 chronic conditions
directly into Epic**



Hyperthyroidism: Yearly Clinical Evaluation, TSH and T4

THE MISSING PIECE TO VALUE- BASED CARE



- Patient engagement is the weak link in delivering value-based care; WakeMed Health and Hospitals's clinical leadership and employer participation can drive better outcomes.
- KBA's proactive care-management team supplements your care-management efforts in outreach to clients.
- WakeMed Health and Hospitals excels in Standards of Care completion rates for the Chronically Ill, improving care and costs.
- Integrating WakeMed Health and Hospitals's managed care with KBA's programs positions WakeMed Health and Hospitals as the value leader.

TRANSPARENT PBM INTEGRATION

- Support of WakeMed Health and Hospitals's retail pharmacy(s)
- Optimize WakeMed Health and Hospitals's 340B drug pricing program
- Allows WakeMed Health and Hospitals to create its own PBM program if desired
- Custom telephone number with personalized scripting and separate call tracking & reporting
- Fully customizable member welcome booklet & materials
- White labeled formulary document and marketing materials

CUSTOM CLIENT SUPPORT:

- Specialized call center capabilities
- Fully customizable plan designs
- Operational access to the platform
- Customized process and functions to promote DTE strategy

CLINICAL FLEXIBILITY:

- Full portfolio of custom formulary offerings – with optimized formulary shield
- Clinical programs that drive towards overall DTE strategy
- Options for additional drug savings by driving members to lower cost alternatives
- Flexibility to customize or layer on utilization management programs

STOP-LOSS INSURANCE

THE KBA EXPERIENCE

- Deep relationships with 20+ stop loss underwriters and carriers
- Over \$150M in annual stop loss premium placement
- Comprehensive pricing and medical service analysis performed for each DTE health plan to ensure proper pricing
- KBA provides the administration required of the stop-loss markets:
 - Bordereau Reporting
 - Claim Reporting and Reconciliation
 - Evaluation of Networks
 - Care Management
 - Initial Underwriting
 - Negotiation of rates and terms
 - Plan Design Actuarial

Tokio Marine

Nationwide

Berkshire

Hathaway

Sun Life

Liberty Mutual

Granular (Google)

Companion Life

Crum & Forster

American Fidelity

RGA

DTE PROGRAM MARKETING & SALES

KBA WORKS WITH YOU TO MANAGE & SELL:

- Branding & Messaging
- Direct & Brokerage Sales Distribution – Focused Sales Representative
- Brochures, Web Site, Media, etc.
- Stop-Loss/Level Funded Down to 20 EE Lives
- Sell Through Established Brokers in Your Service Area

COMMUNITY BASED SALES STRATEGY



Locally based sales rep with strong community ties



Collaboration with DTE provider partners to identify strategic employers for early adoption and sales



Work with select broker distribution partners with aligned incentives and goals



Pipeline reporting and collaboration with our DTE provider partners



Marketing and promotion of the new option in the community

COMPLETE MARKETING SYSTEM

- Customized Branding and Messaging
- Customized Brochures
- Customized Web Site
- Targeted Employer Outreach
- Sales System (Social Media, Email, Text, etc.)



The Key Incentive Plan

Base Medical + Preferred Plus Benefits = Your Deductible Can Simply Vanish

Example: Member with a \$2,500 deductible / \$4,500 max out-of-pocket has a heart attack and cardiac surgery, resulting in a 5-day in-patient stay. In this example, the member would have a \$4,500 OOP with \$5,050 Preferred Plus Benefits for a **net OOP \$0 due to the preferred provider versus \$4,500 OOP exposure at any non-preferred provider.**

BASE MEDICAL BENEFITS*		PREFERRED PLUS BENEFITS		
Deductible		Benefits automatically included in your plan!		
Single	\$2,500		Preferred Provider	Non-Preferred Provider
Family	\$4,500	OP Radiology/X-Ray	\$200	\$0
Coinsurance		OP Diagnostic Labs	\$100	\$0
80% / 20%		Preferred Hospital Stay (\$500/day, 5 days)	\$2,500	\$0
Maximum Out-of-Pocket		Critical Illness Benefit	\$2,000	\$0
Single	\$4,500	Health Screening Benefit	\$125	\$0
Family	\$9,000	Total Employee OOP	\$4,500	\$4,500
Chronic Disease Management Regimens of Care Covered at 100%		Preferred Plus Benefits	\$4,925	\$0
Healthcare Convenience Card to Cover Out-of-Pocket Expenses at 0% interest		Employee Amount Due to Provider	\$0	\$4,500
Health Guardian Account to help members manage their healthcare expenses at Preferred Providers		Additional Benefit Payable to the Health Guardian Account	\$425	\$0

*Base Medical Benefits are for illustration purposes only. Base benefits can be customized to meet client needs.

THE BENEFIT STEERING THE PATIENT TO WAKEMED

What Will this Claim Cost Me?



WakeMed Health and Hospitals patients can have all out-of-pocket liability reduced to \$0!

KEY BENEFIT
ADMINISTRATORS

Explanation of Benefits	
Heart Attack, Cardiac Surgery	
THE CHARGES	
Medical Bills	\$25,000
Healthcare out-of-pocket due to provider	\$4,500
Since your out of pocket maximum has been met, the rest of your eligible charges this year will be paid at 100%	
THE CREDITS	
OP Radiology/X-Ray	\$200
OP Diagnostic Labs	\$100
Preferred Hospital Stay (\$500/day, 5 days)	\$2,500
Critical Illness Benefit	\$2,000
Health Screening Benefit	\$125
Benefit Total	\$4,925
Out of Pocket Paid to Hospital	(\$4,500)
Balance Paid to HGA	\$425

THE KBA HEALTHCARE CONVENIENCE CARD

Creating EPO Steerage in a PPO Network

The Healthcare Convenience Card is eligible to be used only for specific merchant codes.

1. To utilize at Specific Healthcare Providers
 - a. Designated Hospitals
 - b. Urgent Care Facilities
 - c. Physicians
 - d. Eligible Ancillary Providers
2. Out of Pocket Expenses Qualify
3. 0% Interest Rate
4. Employee Determines Repayment Schedules
5. To Qualify: Answer One Simple Question





FINANCIAL PROJECTIONS & ASSUMPTIONS

ESTIMATION OF WAKEMED HEALTH AND HOSPITALS'S 2023 PAYER MIX

Payer Group	Total Inpatient Days	Revenue	Payer Mix
All WakeMed Health and Hospitals	274,316	\$1,853,637K	100%
Medicare	57,881	\$189,127K	21%
Medicaid	45,421	\$348,093K	17%
Other Government Pay	0		
No-Pay, Charity Care	15,314	-\$109,619K	6%
Commercial/Self Pay	155,700	\$1,317,002K	57%

Values in blue are taken from the 2022 Medicare Provider Cost Report (<https://data.cms.gov/provider-compliance/cost-report/hospital-provider-cost-report/data>)

Values in yellow are taken from Definitive Healthcare data.

Total Medicare patient revenue is reported by CMS in the Medicare Provider Analysis and Review Report (MedPAR)

Estimating Payer Mix

- Medicare, Medicaid, and other government payer mixes are estimated as the percentage of inpatient days for each group divided by total inpatient days.
- No-pay and charity care is estimated as the percentage of no-pay/charity care compared to total overall revenue.
- Commercial/self pay is assumed to be the rest of the total payer mix share.

Estimating Total Inpatient Days

- Total inpatient days for 'No Pay' and 'Commercial' are estimated to be the payer mix estimates for "No Pay, Charity Care" and "Commercial/Self Pay" multiplied by the total number of inpatient days.

In other analysis, the outpatient payer mix is assumed to mirror the inpatient payer mix.

ESTIMATION OF MARGIN FOR COMMERCIAL VS. GOVERNMENT PAY

The RAND data for WakeMed Health and Hospitals estimates commercial insurance reimbursements at 257% of Medicare Rates

Estimating Commercial Margin

(Calculated through counterfactual of assuming all CMS/Government pay was paying at commercial rates)

Commercial Revenue	\$1,317,002K
Gov-pay revenue at 257% current rate	\$1,380,067K
Total Revenue	\$2,697,069K
Total Costs	\$1,755,944K
Net Income	\$941,125K
	35%

Estimating Gov-Pay Margin

(Calculated through counterfactual of assuming all commercial pay was paying at Medicare rate)

Commercial Revenue	\$512,671K
Divided by 257%	
Gov-pay revenue	\$537,220K
Total Revenue	\$1,049,891K
Total Costs	\$1,755,944K
Net Income	\$ -706,053K
	-67%

ESTIMATING MARKET SHARE

Hospital Name	Number of Beds	Inpatient Total Charges	Outpatient Total Charges	Inpatient Percent of Market Total	Outpatient Percent of Market Total (Assuming 49.1% of Physicians Employed by Hospitals)
WakeMed Raleigh Campus	734	\$2,738,677.2K	\$2,682,999.3K	19%	6%
WakeMed Cary Hospital	178	\$ 728,197.6K	\$ 860,492.8K	5%	2%
Duke University Hospital	1,097	\$4,596,666.4K	\$7,070,168.6K	32%	17%
North Carolina Specialty Hospital	18	\$ 43,908.5K	\$ 154,154.3K	<1%	<1%
UNC Medical Center	1,009	\$2,930,406.4K	\$4,956,574.7K	21%	12%
UNC Health Johnston - Smithfield	229	\$ 505,523.5K	\$ 922,981.3K	4%	2%
UNC Rex Hospital	775	\$1,661,998.2K	\$3,063,667.7K	12%	7%
Durham VA Medical Center	151	\$ NAK	\$ NAK	NA%	NA%
Duke Regional Hospital	301	\$ 661,920.1K	\$ 932,607.9K	5%	2%
Select Specialty Hospital - Durham	30	\$ 148,609.1K	\$ NAK	1%	NA%
Central Regional Hospital	338	\$ NAK	\$ NAK	NA%	NA%
Holly Hill Hospital	296	\$ 122,944.0K	\$ 4,231.7K	1%	<1%
Triangle Springs	72	\$ 71,075.8K	\$ 30,433.6K	<1%	<1%
Raleigh Oaks Behavioral Health	102	\$ 64,407.9K	\$ 190.4K	<1%	<1%

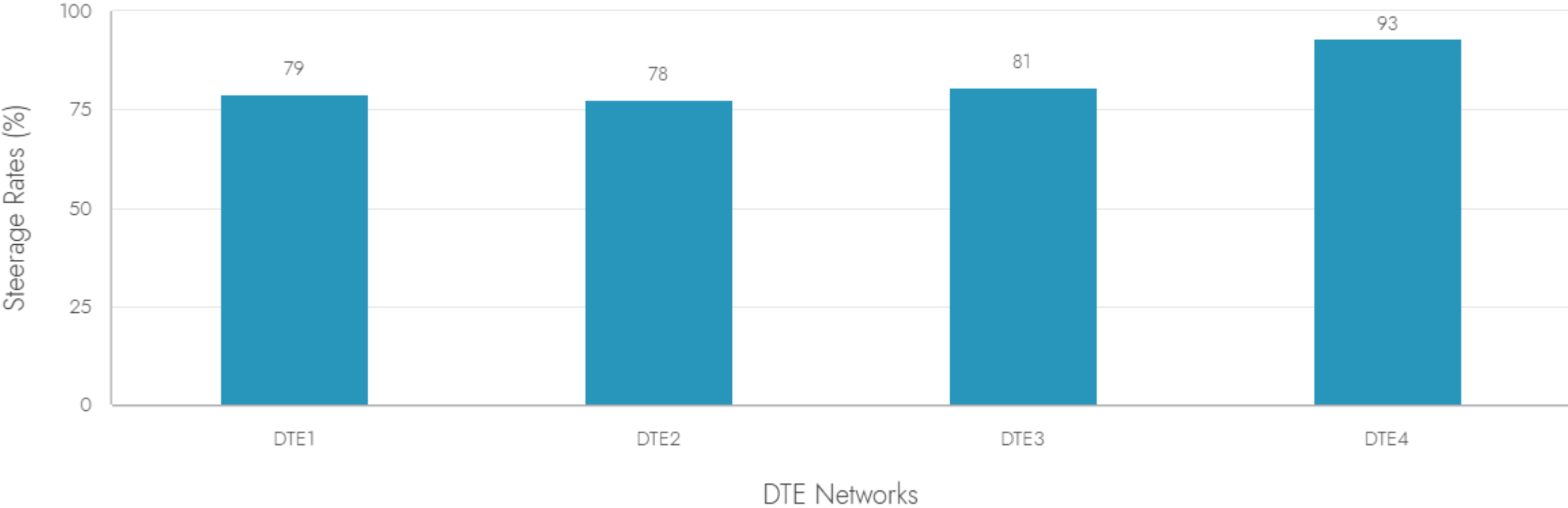
24%
Inpatient
Market
Share

8%
Outpatient
Market
Share

For outpatient market share, it is estimated that only 49.1% of local physicians are employed by area hospitals (Research from the Physician Advocacy Institute:
<https://www.physiciansadvocacyinstitute.org/Portals/0/assets/docs/PAI-Research/PAI-Avalere%20Physician%20Employment%20Trends%20Study%202019-2023%20Final.pdf?ver=uGHF46uIGSeZgYXMKFyYvw%3d%3d>)

Hospital Metrics		
Metric	WakeMed Raleigh Campus	WakeMed Cary Hospital
Total Inpatient Patient Revenue	\$728,488K	\$188,603K
Total Outpatient Patient Revenue	\$713,678K	\$222,868K
Total Medicare Revenue	\$142,036K	\$47,091K
Estimated Medicaid and Other Government-Pay Revenue	\$307,317K	\$40,776K
Estimated Commercial Revenue	\$993,447K	\$323,556K
Total Uncompensated Care	\$92,142K	\$17,477K
Estimated Inpatient Costs	\$699,815K	\$169,843K
Estimated Outpatient Costs	\$685,587K	\$200,699K
Total Operating Expenses	\$1,385,402K	\$370,542K
Net Income	\$57,398K	\$40,880K
Estimated Payer Mix Percent Medicare	19.2%	27.8%
Estimated Payer Mix Percent Medicaid	19.5%	6.1%
Estimated Payer Mix Percent Other Government-Pay	0.0%	0.0%
Payer Mix Percent No Pay	6.0%	4.1%
Estimated Payer Mix Percent Commercial	55.3%	62.0%
Estimated Commercial Profit Margin	35.5%	32.5%
1. Most data is taken from the CMS provider cost report		
2. Medicare costs are taken from the Medicare Provider Analysis and Review (MedPAR)		
3. Data with 'Estimated' in the title are derived from other data points.		
4. Market Share estimates based on total patient revenues with area hospitals. Ambulatory market share based on regional estimates of the percentage of local physicians employed by area hospitals. See Physician Advocacy Institute		

STEERAGE RATES FOR CURRENT DTE NETWORKS



The average Tier 1 Steerage is 83%.

PROJECTED PLAN GROWTH

Year	Employees	Members	Percent of Members Choosing WakeMed Health and Hospitals for Inpatient Care	Percent of Members Choosing WakeMed Health and Hospitals for Outpatient Care
1	600	1,100	60%	50%
2	1,100	2,000	60%	50%
3	1,800	3,200	65%	55%
4	2,800	5,000	67%	58%
5	3,900	7,000	68%	60%
6	5,400	9,700	68%	60%
7	7,200	13,000	69%	61%
8	9,200	16,600	69%	62%
9	11,500	20,700	69%	62%
10	14,000	25,200	70%	63%
11	16,600	29,900	70%	63%
12	19,100	34,400	70%	64%
13	21,400	38,500	71%	64%
14	23,500	42,300	71%	65%
15	25,300	45,500	71%	66%
16	26,700	48,100	72%	66%
17	27,900	50,200	72%	66%
18	28,700	51,700	72%	67%
19	29,400	52,900	72%	67%
20	29,900	53,800	72%	67%

KBA currently estimates that WakeMed Health and Hospitals has 24% inpatient market share, and 8% outpatient market compared to hospitals/clinics in the WakeMed Health and Hospitals service area (see slide 8).

Based on previous DTE experience, KBA expects a significant bump in patient steerage to WakeMed Health and Hospitals.

Additional steerage will occur from additional steerage patient benefits and the patient convenience card.

High estimates start nearly 10% higher than target with low estimates at 10% lower than target

YEAR 4 PROJECTED REVENUE AND PROFIT

Pre-DTE Cohort of 5,000 Employees	Inpatient	Outpatient	Professional	Total
Revenue†	\$ 3,593,533	\$ 2,263,249	\$ 1,056,918	\$ 6,913,699
Net Income‡	\$ 1,006,189	\$ 633,709.6	\$ 295,937	\$ 1,935,836

With DTE	Inpatient	Outpatient	Professional	Total
Revenue°	\$ 9,933,200	\$ 15,577,267	\$ 7,274,451	\$ 32,784,918
Net Income*	\$ 2,781,296	\$ 4,361,635	\$ 2,036,846	\$ 9,179,777

† Assuming proportional revenues from market shares of 24% (inpatient) and 8% (outpatient)

‡ Conservatively assuming a commercial margin of 28% (80% of the estimated commercial margin)

° Assuming 67% inpatient and 58% market steerage based on KBA narrow network DTE experience (see previous slide)

* Conservatively assuming a commercial margin of 28%, with a 10 percentage-point reduction to reflect competitive-edge rates and a 10 percentage-point increase due to lower billing costs and higher patient responsibility collections because of the member convenience card.

Total Increase in Net Income	\$ 7,243,941
Network Access Fee (Revenue) of \$1 PMPM	\$ 60,000
Total Profit Growth	\$ 7,303,941
Minus Implementation Fee (One-Time)	-\$ 0
Minus Sales and Marketing (Annual – Ongoing)	-\$ 180,000
Total	\$ 7,123,941

Target Estimate Assuming 50%+ Within-Plan Steerage and a Commercial Margin of 28%				
All values in thousands of dollars. Modeled With Year 1 Fees and Patient Steerage.				
Metric	500 DTE Covered Lives	1000 DTE Covered Lives	5000 DTE Covered Lives	25000 DTE Covered Lives
Pre-DTE Group Inpatient Revenue	\$359.4	\$718.7	\$3,593.5	\$17,967.7
Pre-DTE Group Inpatient Margin	\$100.6	\$201.2	\$1,006.2	\$5,030.9
DTE Group Inpatient Revenue	\$889.1	\$1,778.1	\$8,890.7	\$44,453.5
DTE Group Inpatient Margin	\$248.9	\$497.9	\$2,489.4	\$12,447.0
DTE Group Incremental Inpatient Revenue	\$529.7	\$1,059.4	\$5,297.2	\$26,485.9
DTE Group Incremental Inpatient Margin	\$148.3	\$296.6	\$1,483.2	\$7,416.0
Pre-DTE Group Outpatient Revenue	\$226.3	\$452.6	\$2,263.2	\$11,316.2
Pre-DTE Group Outpatient Margin	\$63.4	\$126.7	\$633.7	\$3,168.5
DTE Group Outpatient Revenue	\$1,345.0	\$2,689.9	\$13,449.6	\$67,247.8
DTE Group Outpatient Margin	\$376.6	\$753.2	\$3,765.9	\$18,829.4
DTE Group Incremental Outpatient Revenue	\$1,118.6	\$2,237.3	\$11,186.3	\$55,931.6
DTE Group Incremental Outpatient Margin	\$313.2	\$626.4	\$3,132.2	\$15,660.8
Pre-DTE Group Professional Revenue	\$105.7	\$211.4	\$1,056.9	\$5,284.6
Pre-DTE Group Professional Margin	\$29.6	\$59.2	\$295.9	\$1,479.7
DTE Group Professional Revenue	\$628.1	\$1,256.2	\$6,280.8	\$31,404.2
DTE Group Professional Margin	\$175.9	\$351.7	\$1,758.6	\$8,793.2
DTE Group Incremental Professional Revenue	\$522.4	\$1,044.8	\$5,223.9	\$26,119.6
DTE Group Incremental Professional Margin	\$146.3	\$292.5	\$1,462.7	\$7,313.5
Net Margin Less DTE Fees	\$283.8	\$897.6	\$5,808.1	\$30,360.4

Target Estimate Assuming 50%+ Within-Plan Steerage and a Commercial Margin of 28%				
All values in thousands of dollars. Modeled With Year 1 Fees and Patient Steerage.				
Metric	500 DTE Covered Lives	1000 DTE Covered Lives	5000 DTE Covered Lives	25000 DTE Covered Lives
Pre-DTE Group Total Revenue	\$691.4	\$1,382.7	\$6,913.7	\$34,568.5
Pre-DTE Group Total Margin	\$193.6	\$387.2	\$1,935.8	\$9,679.2
DTE Group Total Revenue	\$2,862.1	\$5,724.2	\$28,621.1	\$143,105.6
DTE Group Incremental Total Revenue	\$2,170.7	\$4,341.5	\$21,707.4	\$108,537.1
Incremental Total Revenue by Member	\$4.3	\$4.3	\$4.3	\$4.3
DTE Group Total Margin	\$801.4	\$1,602.8	\$8,013.9	\$40,069.6
DTE Group Incremental Total Margin	\$607.8	\$1,215.6	\$6,078.1	\$30,390.4
Wellness Share	\$6.0	\$12.0	\$60.0	\$300.0
Implementation Fee (One-Time)	\$150.0	\$150.0	\$150.0	\$150.0
Sales and Marketing Annual Fee	\$180.0	\$180.0	\$180.0	\$180.0
Net Margin Less DTE Fees	\$283.8	\$897.6	\$5,808.1	\$30,360.4

High Estimate Assuming 60%+ Within-Plan Steerage and a Commercial Margin of 33%				
All values in thousands of dollars. Modeled With Year 1 Fees and Patient Steerage.				
Metric	500 DTE Covered Lives	1000 DTE Covered Lives	5000 DTE Covered Lives	25000 DTE Covered Lives
Pre-DTE Group Total Revenue	\$691.4	\$1,382.7	\$6,913.7	\$34,568.5
Pre-DTE Group Total Margin	\$193.6	\$387.2	\$1,935.8	\$9,679.2
DTE Group Total Revenue	\$3,406.6	\$6,813.2	\$34,065.9	\$170,329.3
DTE Group Incremental Total Revenue	\$2,715.2	\$5,430.4	\$27,152.2	\$135,760.8
DTE Group Total Margin	\$1,124.2	\$2,248.3	\$11,241.7	\$56,208.7
DTE Group Incremental Total Margin	\$930.6	\$1,861.2	\$9,305.9	\$46,529.5
Wellness Share	\$6.0	\$12.0	\$60.0	\$300.0
Implementation Fee (One-Time)	\$150.0	\$150.0	\$150.0	\$150.0
Sales and Marketing Annual Fee	\$180.0	\$180.0	\$180.0	\$180.0
Net Margin Less DTE Fees	\$606.6	\$1,543.2	\$9,035.9	\$46,499.5

Low Estimate Assuming 40%+ Within-Plan Steerage and a Commercial Margin of 23%				
All values in thousands of dollars. Modeled With Year 1 Fees and Patient Steerage.				
Metric	500 DTE Covered Lives	1000 DTE Covered Lives	5000 DTE Covered Lives	25000 DTE Covered Lives
Pre-DTE Group Total Revenue	\$691.4	\$1,382.7	\$6,913.7	\$34,568.5
Pre-DTE Group Total Margin	\$193.6	\$387.2	\$1,935.8	\$9,679.2
DTE Group Total Revenue	\$2,318.2	\$4,636.4	\$23,182.2	\$115,911.2
DTE Group Incremental Total Revenue	\$1,626.9	\$3,253.7	\$16,268.5	\$81,342.7
DTE Group Total Margin	\$533.2	\$1,066.4	\$5,331.9	\$26,659.6
DTE Group Incremental Total Margin	\$339.6	\$679.2	\$3,396.1	\$16,980.4
Wellness Share	\$6.0	\$12.0	\$60.0	\$300.0
Implementation Fee (One-Time)	\$150.0	\$150.0	\$150.0	\$150.0
Sales and Marketing Annual Fee	\$180.0	\$180.0	\$180.0	\$180.0
Net Margin Less DTE Fees	\$15.6	\$361.2	\$3,126.1	\$16,950.4

COLLABORATION OF STAKEHOLDERS



Key Benefit Administrators

- Full TPA Administration
- Stop-Loss Administration
- True Transparency & Analytics
- Population Health Management
- Utilization & Case Management
- Sales & Marketing
- Transparent PBM
- Website & Mobile App
- Customizable options & plans



WakeMed Health and Hospitals's DTE

- Incremental Revenues and Margin
- Branding & Messaging
- Benefit Plan Integration
- Network Steerage
- Direct Relationship with Self-Funded Employers
- Referral Authorization
- Plan Transparency
- Direct Access to Members



Employer

- Reduced Spend
- "At The Table" with WakeMed Health and Hospitals
- Improved Analytics
- Complete Transparency
- Lower Stop-Loss
- Benefit Plan Stabilization
- Healthier Population



Employee

- Reduced Out of Pocket
- Improved Health
- Better Coordination & Navigation of Care
- Direct Access to the Best Medical Providers

ROLES & RESPONSIBILITIES

SERVICES & ADMINISTRATIVE SYSTEMS PROVIDED BY:	
WakeMed Health and Hospitals	KBA Operations
▪ Tier 1 Provider Network ▪ Provider Recruitment – If Necessary ▪ Network Pricing & Contract Terms ▪ Referral Management Protocols & Approvals ▪ Medical Director Oversight ▪ Practice Improvement Program using KBA/AHDI Data with Assistance from KBA	▪ Network Evaluation – Actuarial Pricing ▪ PPO Wrap & Tertiary Transplant Network, if needed ▪ Claims Administration ▪ Plan Design Development, Testing, and Administration ▪ Employer/Member/Provider Customer Service ▪ Customized Member/Provider/Agent/HR Portal ▪ Billing, Enrollment (Electronic/Paper), and Premium Management ▪ Fulfillment – SPD & Customized ID Cards ▪ COBRA/HIPAA Administration ▪ Out of Network Negotiations ▪ Wrap PPO Repricing ▪ Nurse Coaching for Chronically Ill ▪ Nurse Triage to Physicians and Facilities ▪ Stop-Loss Underwriting, Pricing, and Placement ▪ Stop Loss Claims Administration ▪ Complex Case Management ▪ PBM Administration Interface ▪ Reporting • Utilization & Gaps in Care • Clinically Based Physician Profiling • Referrals • Chronic Disease Management ▪ Employer Health Risk Management Reports ▪ Ancillary Administration – Dental, Vision, Weekly Income, 5500, Life, etc. ▪ Health Savings Account (HSA), Flexible Spending (FSA), Health Reimbursement Account (HRA) Administration ▪ Sales & Marketing Plan • Strategy Development: Branding, Brochures, Web Site • Distribution System Development • Sales Management
KBA Consulting	
▪ Program oversight and integration across the enterprise ▪ Goal development, agreement and tracking ▪ Start-up assessments: market, network, operational and readiness ▪ Draft business plan with financial projections ▪ Facilitate program communication and engagement with WakeMed Health and Hospitals departments including system c-suite, operations, HR, managed care, marketing, strategy and business development	



PRE-IMPLEMENTATION

PROVIDER FILE

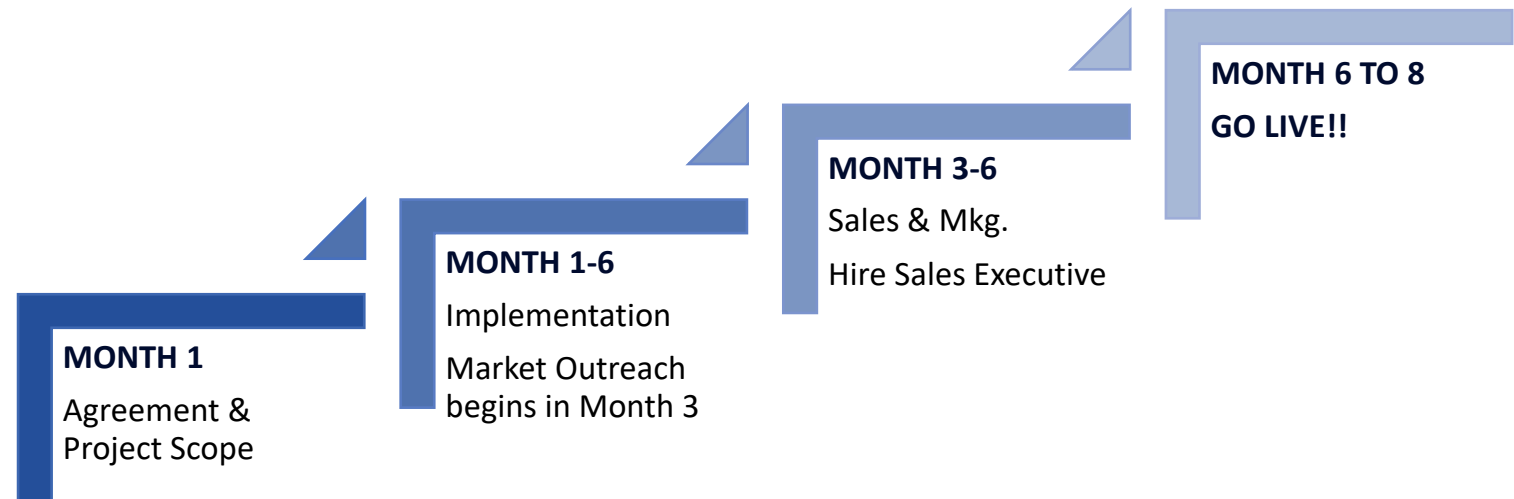
- Determine Gaps in Medical Service Delivery
- Identify Other Key Providers to Potentially Contract with on Behalf of WakeMed Health and Hospitals
- Wrap Network to Cover All Other Provider Types

PRICING

- To Dial in Competitive Stop-Loss Rates and WakeMed Health and Hospitals's Net Revenues/Margins
- Actuarial Evaluation Compared to the Market



IMPLEMENTATION



A PARTNER YOU CAN TRUST



**OUR OBJECTIVES
ARE ALIGNED**



**WE KNOW THE
MARKET**



**COMPLETELY
TRANSPARENT**



**WE HAVE THE
SOPHISTICATION
AND
INFRASTRUCTURE
TO EFFECTIVELY
COMPETE**



Derek Moore, Senior Vice President – KBA
845-283-9973 * derek.moore@keybenefit.com